

CITY GENERAL HOSPITAL

Department of Internal Medicine & Pulmonology

PATIENT DISCHARGE INSTRUCTIONS & MEDICATION GUIDE

Date of Admission:	February 13, 2026	Date of Discharge:	February 18, 2026
Attending Physician:	[REDACTED]	Primary Care Provider:	[REDACTED]
Patient:	[REDACTED]	DOB:	June 22, 1980 (Age 45)
MRN:	3317659	Sex / Weight:	Female / 68 kg
Ward:	4B — General Medicine	Discharge Time:	10:15 AM
Allergies:	Codeine (N&V)	Code Status:	Full Code

DIAGNOSES AT DISCHARGE

PRIMARY:	Community-Acquired Pneumonia (CAP), right lower lobe ICD-10: J18.9 Prevalence: CAP affects ~450 per 100,000 adults annually; ~1.5M hospitalizations/year in the US. S. pneumoniae causes ~30% of all CAP cases. Patients with asthma face 2× the hospitalisation rate. 30-day mortality for hospitalised CAP: ~5–8%.
SECONDARY:	Mild intermittent asthma ICD-10: J45.20 Affects ~262 million people globally (WHO 2023). Complicates respiratory infection and prolongs recovery. Seasonal allergic rhinitis ICD-10: J30.1
COMPLICATIONS:	Hypoxia on admission (SpO2 91% RA) — resolved. No bacteraemia. No effusion.

HISTORY OF PRESENT ILLNESS

Patient is a 45-year-old female with mild intermittent asthma and seasonal allergic rhinitis who presented to the Emergency Department on February 13, 2026. Patient reported a 5-day history of productive cough with yellowish sputum, fever with rigors (Tmax 39.2°C / 102.6°F), right-sided pleuritic chest discomfort, and dyspnoea on exertion. No haemoptysis. Non-smoker. COVID-19 and influenza rapid antigen tests negative. CXR: right lower lobe consolidation with air bronchograms. SpO2 91% RA at triage. Admitted for IV antibiotics and oxygen.

SUMMARY OF HOSPITAL COURSE

Day 1 (Feb 13):	Admitted ward 4B. IV Ceftriaxone + IV Azithromycin commenced. O2 at 2L/min NC. Sputum and blood cultures taken
Day 2 (Feb 14):	Fever persisting 38.3°C. Blood cultures: no growth at 48h. O2 unchanged. Incentive spirometry started.
Day 3 (Feb 15):	Fever resolved. Weaned to room air — SpO2 95–97% maintained. Sputum: S. pneumoniae (sensitive). Switched to oral Azithromycin
Day 4 (Feb 16):	Repeat CXR: improving consolidation. Albuterol nebulizers transitioned to MDI. Appetite improved. Ambulatory.
Days 5–6 (Feb 17–18):	Afebrile >48h. SpO2 97% RA. All oral medications tolerated. Discharge planned and education completed.

■ PENDING AT DISCHARGE:

- 1. Repeat CXR — to be arranged at GP follow-up (not yet available).
 - 2. Spirometry / pulmonary function test — outpatient referral placed (see Follow-Up section).
- GP will be notified of results. Contact GP clinic if not heard within 2 weeks.

CLINICAL DOCUMENTATION — INTERNAL HOSPITAL RECORDS (not for patient use)

ADM DIAG: CAP right lower lobe. TRIAGE CAT: 3. ED DISPO: Admit — Internal Medicine. CONSULTS: Physiotherapy (Day 2), Pharmacy (medication reconciliation Day 1). VITALS ON ADMISSION: BP 138/84 | HR 102 | RR 22/min | SpO2 91% RA | Temp 39.2°C | Wt 68 kg | Ht 163 cm | BMI 25.6 | Pain 7/10 VAS.

LABS (Feb 13, 07:42): WBC 14.2 (H) | Neutrophils 11.8 (H) | Hgb 128 | Plt 287 | Na 138 | K 3.9 | Cr 72 umol/L | eGFR >90 | Glucose 6.4 | CRP 142 (H) | Procalcitonin 1.8 (H) | Lactate 1.4 | ALT 28 | AST 31 | ALP 74 | Bili 12 | Albumin 36.

MICROBIOLOGY: Blood cultures x2 — NO GROWTH at 120h (final). Sputum C&S: Heavy growth *S. pneumoniae* — Pen MIC 0.06 (S), Azithromycin MIC ≤0.25 (S). Urine *Legionella* Ag: NEG. Urine *Pneumococcal* Ag: POS. COVID-19 PCR: NEG. Influenza A/B: NEG.

IMAGING: CXR Feb 13 — RLL consolidation with air bronchograms, no effusion, no pneumothorax. CXR Feb 16 — Improving RLL consolidation, approx 40% reduction. No new findings. CT Chest not performed (not indicated per CAP guidelines).

ECG (Feb 13): Sinus tachycardia 102 bpm. Normal axis. No ST changes. QTc 418ms.

IV ACCESS: R antecubital PIV 18G (Feb 13), removed Feb 15 on oral transition. O2: 2L NC (Feb 13–15) weaned to RA Feb 15. SpO2 95–97% on RA from Feb 15.

NURSING NOTES: Ambulatory with assistance. Appetite improving. No falls. Compression stockings applied. Padua DVT Score 2 (low risk). Incentive spirometry encouraged QID. Fluid balance: +820mL Day 1, neutral Days 2–5.

MEDS IN HOSPITAL: IV Ceftriaxone 1g/day (Feb 13–15) | IV Azithromycin 500mg (Feb 13–15) | PO Azithromycin 250mg (Feb 15–18) | Paracetamol 1g QID PRN | Prednisone 40mg daily (Feb 16–18, Day 1 of 3-day course) | Salbutamol neb 2.5mg TID (Feb 13–14) → MDI Day 3 | Cetirizine 10mg daily (home med continued) | Enoxaparin 40mg SC (VTE prophylaxis Days 1–3, ceased on improved mobility).

SOCIAL: Non-smoker. Occasional alcohol (1–2 units/week). Lives alone (apartment, elevator). Employed full-time. Emergency contact: Sister (details on file).

FUNCTIONAL STATUS AT DISCHARGE: Independent with ADLs. No home care required. Medication education completed. GP letter transmitted electronically.

ICD-10: J18.9 (CAP) | J45.20 (mild persistent asthma) | J30.1 (allergic rhinitis).

DRG: MDC 04 — Respiratory. DRG 194 — Simple Pneumonia w/ MCC. ALOS benchmark 4.1 days. Actual LOS 5 days. Variance +0.9 days (documented: persistent hypoxia Day 3).

CONSENT: Admission consent signed Feb 13. No procedural consents required.

DISCHARGE MEDICATIONS — COMPLETE GUIDE

IMPORTANT: Read all medication instructions before leaving. If tablets look different from what is described here, do NOT take them — contact the pharmacy first.

#	Medication	Dose	Frequency	Duration	Route	Status
1	Azithromycin (Z-Pack)	250mg	Once daily	4 days (last: Feb 22)	By mouth	NEW
2	Prednisone	40mg	Once daily (morning, with food)	3 days (last: Feb 21)	By mouth	NEW
3	Salbutamol / Albuterol MDI	2 puffs (90mcg/puff)	Every 4–6h as needed	Until follow-up	Inhaled	Existing
4	Cetirizine (Zyrtec)	10mg	Once daily	Ongoing	By mouth	Existing

Medication Details with Visual Guide

Illustrations show approximate appearance. Confirm with pharmacist if your tablets look different.



Azithromycin 250mg tablet

Azithromycin (Z-Pack) 250mg tablet

How to take: 1 tablet once daily at the same time each day. May be taken with or without food. Complete ALL 4 remaining doses — do not stop early even if feeling better.

Common side effects: nausea • diarrhea • stomach cramps • headache • mild dizziness

■ **Rare but serious: irregular heartbeat (QT prolongation) — seek emergency help for racing or skipping heartbeat.**

■ **May interact with antacids containing aluminium/magnesium — take 1h before or 2h after.**

■ **Contact GP if diarrhoea is severe or contains blood (rare C. difficile risk).**

✗ **Do NOT take if: known allergy to azithromycin, erythromycin, or any macrolide antibiotic.**

✗ **Do NOT take if: currently taking heart medications (amiodarone, sotalol) — check with doctor first.**



Prednisone 40mg tablet

Prednisone 40mg tablet

How to take: 1 tablet once daily in the MORNING with food or milk. Take for exactly 3 days only. Do not take on an empty stomach.

Common side effects: increased appetite • difficulty sleeping • mild mood changes (irritability) • elevated blood sugar (monitor if diabetic) • stomach irritation without food • facial flushing

■ **May temporarily raise blood glucose — monitor more frequently if diabetic.**

■ **Avoid contact with anyone who has chickenpox or shingles while taking steroids.**

■ **May reduce vaccine effectiveness — inform clinicians before receiving vaccinations.**

✗ **Do NOT take if: active, untreated systemic fungal infection.**

✗ **Do NOT take if: known allergy to corticosteroids.**



Salbutamol 90mcg per actuation

Salbutamol / Albuterol MDI Inhaler 90mcg per actuation

How to take: Shake well before use. Breathe out fully, then inhale 2 puffs slowly. Hold breath 10 seconds after each puff. Wait 1 minute between puffs. Use only as needed — NOT on a fixed schedule.

Common side effects: shakiness / tremor (most common) • fast heartbeat • headache • throat irritation — rinse mouth after use • mild nervousness

■ **Using >8 puffs in 24h without relief — seek urgent medical attention.**

■ **Not relieved within 20 min of 2 puffs during an attack — call 000 / 911.**

■ **Overuse can mask worsening asthma — see GP if needing it daily.**

✗ **Do NOT take if: known hypersensitivity to salbutamol.**



Cetirizine 10mg
tablet

Cetirizine (Zyrtec) 10mg tablet

How to take: 1 tablet once daily at the same time each day. Can be taken with or without food. Continue your usual pre-admission dose.

Common side effects: drowsiness (~10–15% of users) • dry mouth • headache • fatigue ■ **May cause drowsiness — avoid driving or machinery if affected.**

■ **Alcohol intensifies drowsiness — avoid while taking this medication.**

✗ **Do NOT take if: severe kidney disease — dose adjustment may be needed.**

✗ **Do NOT take if: known allergy to cetirizine or levocetirizine.**

■ **GENERAL MEDICATION SAFETY:**

- Do NOT take ibuprofen or aspirin while on Azithromycin (stomach bleeding risk).
- Do NOT share medications. Do NOT take other paracetamol-containing products simultaneously.
- Missed dose: take as soon as remembered — if nearly time for next dose, skip the missed one.
- Store at room temperature, away from moisture. Keep out of reach of children.
- Return unused or expired medications to the pharmacy for safe disposal.

FOLLOW-UP CARE PLAN

You have **two follow-up contacts** from this admission. Do not miss these appointments — they confirm your recovery and guide your ongoing asthma management.

APPOINTMENT 1 — Mandatory: Post-Pneumonia GP Review

When:	Within 7–10 days of discharge — on or before February 28, 2026. Call your GP clinic now to book. Do not wait for them to contact you.
Phone:	(555) 204-9300 — GP Patient Line
Purpose:	Review recovery, confirm antibiotic course completed, assess symptoms, obtain chest X-ray results, update asthma action plan.
Please bring:	• All current medication bottles (including remaining Azithromycin / Prednisone) • Your salbutamol inhaler • This discharge summary • A list of any new symptoms noticed at home
What to expect:	GP will listen to your lungs and check oxygen levels. A repeat chest X-ray will be arranged at a radiology clinic — this is standard after pneumonia. GP may also discuss starting a daily preventer inhaler for your asthma.
If GP unavailable:	Attend an urgent care clinic or return to City General Hospital ED — bring this document.

APPOINTMENT 2 — Referral: Outpatient Respiratory Clinic (4–6 weeks)

When:	To be confirmed within 4–6 weeks. You will receive a letter or call. If not contacted by March 7, 2026, call: (555) 204-7700 — Respiratory Outpatient Booking.
Purpose:	Full pulmonary function testing (spirometry) to assess lung function post-pneumonia and formally evaluate your asthma. The respiratory team will advise on long-term preventer therapy.
Please bring:	• All current inhalers • Symptom diary if kept • This discharge summary and GP follow-up letter
What to expect:	Non-invasive breathing tests — blow forcefully into a machine several times. No needles. Takes approximately 45–60 minutes. Results reviewed same day.

Getting Your Prescriptions Filled

- Fill Azithromycin and Prednisone prescriptions TODAY — start immediately.
- Your inhaler and Cetirizine supply should be sufficient. Ask GP for repeats at follow-up.
- Tell the pharmacist about your Codeine allergy.

WARNING SIGNS — WHEN TO SEEK HELP

■ CALL 000 / 911 OR GO TO EMERGENCY IMMEDIATELY:

- Severe shortness of breath or difficulty breathing at rest
- Lips, fingernails, or fingertips turning blue or grey (cyanosis)
- Sharp, constant chest pain — especially spreading to arm or jaw
- Fever above 39.5°C / 103°F not responding to paracetamol
- Coughing up significant amounts of blood
- Sudden confusion or loss of consciousness

■ CALL YOUR GP (business hours) if you have:

- Fever above 38°C after 48 hours on antibiotics at home
- Cough worsening or producing green / brown mucus
- Wheezing not relieved by inhaler within 20 minutes
- No noticeable improvement after **3 days** at home
- New leg swelling — one leg more swollen than the other (possible clot)
- Rash or new medication reaction

RECOVERY AT HOME

Rest & Activity

- Fatigue is normal and may persist 2–4 weeks — this does not mean you are not recovering.
- Do not return to work or strenuous activity until cleared by GP at follow-up.
- Start with short walks at home; increase distance gradually as tolerated.
- Sleep with head elevated on 1–2 extra pillows to ease overnight breathing.
- Avoid cigarette smoke, dust, and chemical fumes during recovery.

Breathing Exercises — 3 times daily

Step 1	Sit upright in a chair, feet flat on floor.
Step 2	Inhale slowly through your nose over 4 seconds — feel your belly rise.
Step 3	Hold 2 seconds.
Step 4	Exhale slowly through your mouth over 6 seconds.
Step 5	Rest 2 normal breaths; repeat 10 times per session.
Step 6 (Huff Cough)	Take a deep breath and say 'HUF' forcefully (not a cough). This loosens mucus. Follow with a controlled cough into a tissue.

Diet & Fluids

- Drink 6–8 glasses of water or warm fluids daily — helps thin mucus.
- Eat small, frequent meals — reduced appetite after pneumonia is normal.
- Avoid alcohol during the antibiotic course.

■ PATIENT EDUCATION — UNDERSTANDING YOUR HEALTH

A. What Is Pneumonia and Will My Lungs Fully Recover?

What happened: Pneumonia is an infection of lung tissue where the air sacs fill with fluid or pus, reducing oxygen transfer into the blood. Your right lower lobe was affected by *Streptococcus pneumoniae* — a bacterium, which is why antibiotics worked. Viral pneumonia does not respond to antibiotics.

Recovery outlook: Most healthy adults recover fully within 4–8 weeks. A lingering cough and tiredness for several weeks are normal — the lungs are still clearing residual inflammation even after the infection is gone. Your follow-up X-ray will confirm the consolidation shadow has cleared. No long-term lung damage is expected.

B. Your Asthma and How It Relates to This Admission

Asthma causes persistently inflamed airways, making it harder to clear respiratory infections. Prednisone was given for 3 days specifically to reduce asthma-related airway inflammation — it is not a long-term treatment. Your respiratory clinic appointment will assess whether you need a daily preventer inhaler (inhaled corticosteroid). Do not start one without specialist guidance.

C. Preventing Future Respiratory Infections

- **Pneumococcal vaccine:** A vaccine against *S. pneumoniae* exists. Adults with asthma are strongly recommended to receive it. Discuss eligibility with your GP at follow-up.
- **Annual flu vaccine:** Influenza significantly increases the risk of secondary bacterial pneumonia. Strongly recommended for anyone with asthma.
- **Hand hygiene:** Wash hands ≥ 20 seconds with soap and water after public spaces and before eating.
- **Avoid smoking:** Smoking damages the lungs' natural defenses, increasing susceptibility to chest infections.

D. Recognising a Medical Emergency

BREATHING	Severe shortness of breath at rest — cannot speak in full sentences Rapid breathing >25 breaths/minute Blue or grey lips, tongue, or fingertips (cyanosis)
CHEST	Chest pain or pressure lasting >2 minutes, spreading to arm, jaw, or back Heart racing, pounding, or irregular
NEUROLOGICAL	Sudden confusion, slurred speech, facial drooping, arm weakness Seizure or loss of consciousness
INFECTION	Fever $>39.5^{\circ}\text{C}$ with shaking chills and confusion Feeling faint with rapid heart rate (possible sepsis)

E. Emotional Recovery After a Hospital Admission

It is common to feel anxious or low in mood after an acute illness requiring hospitalization. If you experience persistent anxiety, low mood, or significant difficulty sleeping beyond 1–2 weeks at home, raise this with your GP at follow-up. Support is available.

This document was prepared by the medical and nursing team at [REDACTED] on the date of discharge. It does not replace professional medical advice. A copy has been sent electronically to your registered GP. In any life-threatening emergency, call emergency services immediately.

Discharging Physician: [REDACTED], MD — Pulmonology & Internal Medicine

Discharge Nurse: [REDACTED], RN — Ward 4B

Date / Time: February 18, 2026 — 10:15 AM

Patient Signature: _____ (confirms receipt and understanding)